

PRODUCT LITERATURE

PRONERGAN TABLET 12.5 MG

Each Tablet contains

Promethazine HCl BP 12.5 mg

Product Description

White coloured round flat tablet with score on one and "TPI" and "J/61" embossed on the other side

Pharmacodynamics properties

Mechanism of action/Effect:

Antihistaminic (H₁-receptor): Antihistamines used in the treatment of allergy act by competing with histamine for H₁-receptor sites on effector cells. They thereby prevent, but do not reverse, responses mediated by histamine alone. Antihistamines antagonize, in varying degrees, most of the pharmacological effects of histamine, including urticaria and pruritus. In addition, the anticholinergic actions of most antihistamines provide a drying effect on the nasal and oral mucosa.

Antiemetic; Anti vertigo: The mechanism by which some antihistamines exert their antiemetic, anti-motion sickness and antivertigo effects is not precisely known but may be related to their central anticholinergic actions. They diminish vestibular stimulation and depress labyrinthine function. Activity on the medullary chemoreceptive trigger zone may also be involved in the antiemetic effect.

Sedative – hypnotic: Most antihistamines cross the blood-brain barrier and produce sedation due to inhibition of histamine N-methyltransferase and blockage of central histaminergic receptors. Antagonism of other central nervous system receptor sites, such as those for serotonin, acetylcholine, and alpha-adrenergic stimulation, may also be involved. Phenothiazines are thought to cause indirect reduction of stimuli to the brain stem reticular system.

Pharmacokinetics properties

Absorption:

Well absorbed after oral administration.

Protein binding:

Promethazine: High (65-90%).

Biotransformation:

Hepatic ; some renal.

Half-life:

Elimination: Promethazine: 7 to 14 hours.

Onset of action:

Oral:

15 to 60 minutes.

Duration of action:

Promethazine: 4 to 6 hours; may persist for up to 12 hours.

Elimination:

Renal - Most of the antihistamines studied are excreted as metabolites within 24 hours.

Indication

Management of allergic condition, motion sickness, nausea and vomiting in anesthesia.

Recommended Dose

Adult: 1 tablet, 4 times a day

6 - 12 years. : 1/2 - 1 tablet, 2 or 3 times a day

Route of Administration

Oral

Contraindications

Risk-benefit should be considered when the following medical problems exist

-Bladder neck obstruction or

-Prostatic hypertrophy, symptomatic or

-Urinary retention, predisposition to (anticholinergic effects may precipitate or aggravate urinary retention)

-Bone marrow depression (increased risk of leukopenia and agranulocytosis)

-Cardiovascular disease (increased risk of transient hypotension)

-Coma (may be exacerbated)

-Epilepsy (parenteral administration of promethazine may increase severity of seizures)

-Glaucoma, angle-closure or predisposition to (mydriatic effect resulting in increased intraocular pressure may precipitate an attack of angle-closure glaucoma)

-Glaucoma, open-angle (mydriatic effect may cause a slight increase in intraocular pressure; glaucoma therapy may need to be adjusted)

-Hepatic function impairment (metabolism may be decreased; higher serum concentrations may increase sensitivity to CNS effects)

-Jaundice (may be exacerbated with parenteral administration of Promethazine)

-Reye’s syndrome (extrapyramidal symptoms that may be produced by parenteral administration of promethazine may be confused with CNS signs of Reye’s syndrome

Sensitivity to the antihistamine used.

Caution is recommended when phenothiazine-derivative antihistamines are used, since their antiemetic action may impede diagnosis of such conditions as appendicitis and obscure signs of toxicity from overdosage of other drugs.

Warnings and Precautions

Cross-sensitivity and/or related problems

Patients sensitive to other phenothiazines may be sensitive to Methdilazine, Promethazine, and Trimeprazine also.

Carcinogenicity/Tumorigenicity/Mutagenicity

Long-term animal studies to evaluate the carcinogenic, tumorigenic, or mutagenic potential of most antihistamines have not been performed.

Pediatrics

Use is not recommended in newborn or premature infants because this age group has an increased susceptibility to anticholinergic side effects, such as central nervous system (CNS) excitation, and an increased tendency toward convulsions.

A paradoxical reaction characterized by hyperexcitability may occur in children taking antihistamines.

The use of phenothiazine-derivative antihistamines is not recommended in infants up to 3 months of age, because of the possible absence or deficiency of detoxifying enzyme and inefficient renal function usually noted in this age group. Also, increased susceptibility to dystonias has been reported in newborn or premature infants, acutely ill or dehydrated children, and children with acute infections who have received phenothiazine medication.

Some studies have associated the use of promethazine with sudden infant death syndrome (SIDS) and with an increase in infant sleep apnea. Until more studies have been performed to confirm this potential risk, phenothiazine derivatives should not be used in children up to 2 years of age.

In children with signs and symptoms suggestive of Reye’s syndrome, phenothiazine-derivative antihistamines should not be used since the extrapyramidal symptoms that may occur, especially after parenteral administration of large doses, may be confused with the CNS signs of this syndrome, thus making diagnosis difficult.

Adolescents

In adolescents with signs and symptoms suggestive of Reye’s syndrome, phenothiazine-derivative antihistamines should not be used since the extrapyramidal symptoms that may occur, especially after parenteral administration of large doses, may be confused with the CNS signs of this syndrome, thus making diagnosis difficult.

Geriatrics

Dizziness, sedation, confusion, and hypotension may be more likely to occur in geriatric patients taking antihistamines.

A paradoxical reaction characterized by hyperexcitability may occur in geriatric patients taking antihistamines.

Geriatric patients are especially susceptible to the anticholinergic side effects, such as dryness of the mouth and urinary retention (especially in males), of the antihistamines. If these side effects occur and continue or are severe, the medication should probably be discontinued.

Extrapyramidal signs, especially Parkinsonism, akathisia, and persistent dyskinesia, may also be more likely to occur in geriatric patients, especially at the higher doses or with parenteral administration.

Dental

Prolonged use of antihistamines may decrease or inhibit salivary flow, especially in middle-aged or elderly patients, thus contributing to the development of caries, periodontal disease, oral candidiasis, and discomfort.

Involuntary Orofacial muscle movement may result from extrapyramidal effects. These involuntary movements may result in occlusal adjustments, bite registrations, and treatment for bruxism being less reliable.

Interactions with Other Medicaments

- Alcohol or
- CNS depression producing medications, other (concurrent use may potentiate the CNS depressant effects of either these medications or antihistamines; also, concurrent use of Maprotiline or tricyclic antidepressants may potentiate the anticholinergic effects of either antihistamines or these medications)
- Amphetamines (concurrent use may decrease stimulant effects of amphetamines since phenothiazine derivatives produce alpha-adrenergic blockade)
- Anticholinergics or other medications with anticholinergic activity (anticholinergic effects may be potentiated when these medications are used concurrently with antihistamines; patients should be advised to report occurrence of gastrointestinal problems promptly since paralytic ileus may occur with concurrent therapy)
- Anticonvulsants, including barbiturates (phenothiazine derivatives may lower the convulsion threshold; dosage adjustment of anticonvulsant medications may be necessary; potentiation of anticonvulsant effects does not occur)
- Appetite suppressants (concurrent use with phenothiazine derivatives may antagonize the anorectic effect of appetite suppressants)
- Beta-adrenergic blocking agents, especially propranolol (concurrent use with phenothiazine derivatives may result in increased plasma concentration of each medication because of inhibition of metabolism; this may result in additive hypotensive effects, irreversible retinopathy, cardiac arrhythmias, and tardive dyskinesia)
- Bromocriptine (concurrent use may increase serum prolactin concentrations and interfere with effects of Bromocriptine; dosage adjustments of bromocriptine may be necessary)
- Dopamine (concurrent use may antagonize peripheral vasoconstriction produced by high doses of dopamine because of the alpha-adrenergic blocking action of phenothiazine derivatives)
- Ephedrine or
- Metaraminol or
- Methoxamine (alpha-adrenergic blocking action of phenothiazine derivatives may decrease the pressor response to these medications when used concurrently)
- Epinephrine (alpha-adrenergic effects of epinephrine may be blocked when it is used concurrently with phenothiazine derivatives, possibly resulting in severe hypotension and tachycardia)
- Extrapyramidal reaction causing medications, other (concurrent use with phenothiazine derivatives may increase the severity and frequency of extrapyramidal effects)
- Guanadrel or Guanethidine (neuronal uptake of these medications may be inhibited when they are used concurrently with phenothiazine derivatives, causing a decrease of their antihypertensive effect)
- Hepatotoxic medications, other (concurrent use of phenothiazine derivatives with other hepatotoxic medications may increase the potential for hepatotoxicity; patients, especially those on prolonged therapy or with a history of liver disease, should be carefully monitored)
- Hypotension-producing medications, other (concurrent use with phenothiazine derivatives may produce additive hypotensive effects)
- Levodopa (antiparkinsonian effects of levodopa may be inhibited when used concurrently with phenothiazine derivatives because of blockade of dopamine receptors in the brain; levodopa has not been shown to be effective in phenothiazine-induced Parkinsonism)
- Metrizamide, intrathecal (concurrent use with phenothiazine derivatives may lower the seizure threshold; phenothiazine derivatives should be discontinued at least 48 hours before, and not resumed for at least 24 hours following, myelography)
- Monoamine oxidase (MAO) inhibitors, including furazolidone, procarbazine, and selegiline (concurrent use of MAO inhibitors with antihistamines in general may prolong and intensify the anticholinergic and CNS depressant effects of antihistamines; concurrent use of MAO inhibitors with phenothiazine-derivative antihistamines may increase the risk of hypotension and extrapyramidal reactions; concurrent use is not recommended)
- Ototoxic medications (concurrent use with antihistamines may mask the symptoms of ototoxicity such as tinnitus, dizziness, or vertigo)
- Quinidine (concurrent use with phenothiazine-derivative antihistamines may result in additive cardiac effects)
- Riboflavin (requirements for riboflavin may be increased in patients receiving phenothiazine-derivative antihistamines)

Pregnancy and Lactation

Pregnancy/Reproduction

Pregnancy:

Phenothiazines have been reported to cause jaundice and extrapyramidal symptoms in infants whose mothers received these medications during pregnancy.

For Promethazine

Adequate and well-controlled studies in humans have not been done. However, promethazine taken within 2 weeks prior to delivery may inhibit platelet aggregation in the newborn.

Studies in rats with doses 2.1 to 4.2 times the maximum recommended human daily dose have not shown that promethazine causes adverse effects on fetal development.

FDA Pregnancy Category C.

Breast-feeding

Small amounts of antihistamines may be distributed into breast milk; use is not recommended in nursing mothers because of the risk of adverse effects, such as unusual excitement or irritability, in infants.

Antihistamines may inhibit lactation because of their anticholinergic actions.

Some studies have indicated that the use of promethazine in children up to 2 years of age may be associated with the sudden infant death syndrome (SIDS) and an increase in sleep apnea, thus possibly increasing the risk to the nursing infant. Therefore, the use of phenothiazine-derivative antihistamines by nursing mothers should be discouraged until more studies have been performed to confirm the potential risk to the nursing infant.

Side Effects

Those indicating need for medical attention
Incidence less frequent or rare
Blood dyscrasias (sore throat; fever; unusual bleeding or bruising; unusual tiredness or weakness)
Those indicating need for medical attention only if they continue or are bothersome
Incidence more frequent
Drowsiness
Thickening of mucus

Note: Sedative effects are more pronounced with promethazine and less pronounced with trimeprazine and methdilazine, in that order.

- Incidence less frequent or rare
- Blurred vision or any change in vision
 - Burning or stinging of rectum for promethazine rectal dosage form only
 - Confusion
 - Difficult or painful urination
 - Dizziness
 - Dryness of mouth, nose, or throat
 - Hypotension (feeling faint)
 - Increased sweating
 - Loss of appetite
 - Paradoxical reaction (nightmares; unusual excitement, nervousness, restlessness, or irritability)
 - Photosensitivity (increased sensitivity of skin to sun)
 - Ringing or buzzing in ears
 - Skin rash
 - Tachycardia (fast heartbeat)

Note: Confusion; difficult or painful urination; dizziness; drowsiness; and dryness of mouth, nose, or throat are more likely to occur in the elderly.
Nightmares, unusual excitement, nervousness, restlessness, or irritability are more likely to occur in children and elderly patients.

Symptoms of overdose

Clinical effects of overdose
The following effects have been selected on the basis of their potential clinical significance (possible signs and symptoms in parentheses where appropriate) not necessarily inclusive:

- Acute and chronic
- Anticholinergic effects (clumsiness or unsteadiness; severe drowsiness; severe dryness of mouth, nose, or throat; flushing or redness of face; shortness of breath or troubled breathing)
 - CNS depression (severe drowsiness)
 - CNS stimulation (hallucinations; seizures; trouble in sleeping)
 - Extrapyramidal effects (muscle spasms, especially of neck and back; restlessness; shuffling walk; tic-like [jerky] movements of head and face; trembling and shaking of hands)
 - Hypotension, severe (feeling faint)

Note: Anticholinergic and CNS stimulant effects are more likely to occur in children with overdose. Hypotension may also occur in the elderly at usual doses.

Treatment of overdose

Since there is no specific antidote for overdose with antihistamines, treatment is symptomatic and supportive with possible utilization of the following:

To decrease absorption:
Induction of emesis (syrup of ipecac recommended); however, precaution against aspiration is necessary, especially in infants and children.
Gastric lavage (isotonic or 0.45% sodium chloride solution) if patient is unable to vomit within 3 hours of ingestion.

To enhance elimination:
Saline cathartics (milk of magnesia) are sometimes used.

Specific treatment:
Vasopressors to treat hypotension; however, epinephrine should not be used since it may further lower blood pressure.
Anticholinergic antiparkinson agents, diphenhydramine, or barbiturates, to control extrapyramidal reactions.
Precaution against use of stimulants (analeptic agents) because they may cause seizures.
Supportive care:
Oxygen and intravenous fluids.

Packing

Packed in blister-packs of 10 x 10's / Box

Pharmaceutical Precautions

Store below 30°C in a dry place, protected from direct light.
Keep away from reach of children

Expiry Date

3 years from date of manufacturer

Name and Address of Manufacturer:
TERAPUTICS SDN. BHD. (590500-W)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Name and Address of Product Registration Holder/ Distributor:
ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Malaysian Drug Registration No. MAL19872194ACZ

Date of Revision: 27/12/2019